



Corporate Office : HY PATHO LAB

CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028

7718962488

info@hyatholab.in

www.hyatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : SUMIT AGARWAL

Age : 48 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. DR.NAVNEET GUPTA

Registered on : 31-08-2026 12:05 PM

ARYA DIGNOSTIC LAB

Reg No : HL5701831905 / GUJ01

Received on : 01-09-2026 10:01 AM

Barcode : GO224636

Reported on : 01-09-2026 01:09 PM



INV : HIV 1 & 2 Antibody -Rapid

SAMPLE : Serum

HIV 1 & 2 ANTIBODY -RAPID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HIV -1 , Rapid Method: rapid	NON REACTIVE	.	. - .
HIV -2 , Rapid Method: rapid	NON REACTIVE	-	---

INTERPRETATION

Two type of human deficiency viruses, called HIV-1 and HIV-2, have been identified to date. Various subtypes of the known HI-Viruses have been described, each of which has a different geographical distribution. HIV-1 can be divided into 3 distantly related groups :group M(for main),group N (for non-M, non-O) and group O (for outlier).Based on their gentic relationship,at least 9 different subtypes (A to D, F to H,J,K) have been identifiedwithin HIV-1 group M.Antibodies to HIV proteins, indicating the presence of an HIV infection, can be found in the serum usually 6-12 weeks after infection.Due to differences in the sequence of immunodominant epitopes, especialt in the envelope proteins HIV-1 group M HIV-1 group O and HIV-2,specific antigens are necessary to avoid failure in the detection of an HIV infection by immunoassays.

Repeatedly reactive samples must be confirmed according to recommended confirm-atory algorithms.

Confirmatory tests include Western Blot HIV RNA tests.

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



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Toll Free No. : 18001030287

REPORT

PT Name : MR. AAYUSH YADAV

Ref By : DR. DR. SANTOSH YADAV

Reg No : HL2501832638 / HYMH/KDV/MATOSHREE

Barcode : HDM002404

INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

Age : 23 Year | Sex : Male

Registered on : 02-09-2026 04:56 PM

Received on : 02-09-2026 06:37 PM

Reported on : 03-09-2026 12:21 AM

SAMPLE COLLECTED AT :

MATOSHREE DIAGNOSTICS



SAMPLE : Serum

HIV 1 & 2 ANTIBODY ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HIV -1-2 Method: ELISA	0.12	ODRatio	0.0 - 0.90
Reference Range			
Negative : <= 0.90			
Equivocal : 0.91-1.10			
Positive : >= 1.11			

Please correlate with clinical conditions.

~~End of report~~

NRahate

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Toll Free No. : 18001030287

REPORT

PT Name : MRS SABIRA SHAH

Age : 27 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. NISHAT AFRIN KHAN

Registered on : 31-08-2026 12:35 PM

Reg No : HL3901831934 / MH771

Received on : 31-08-2026 04:56 PM

Barcode : HEN154194

Reported on : 31-08-2026 05:32 PM

INV : Hepatitis C Virus (HCV Rapid)



SAMPLE : Serum

HEPATITIS C VIRUS (HCV RAPID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS C VIRUS (HCV RAPID)	NON REACTIVE	.	. - .

Method: .

interpretation

- Hepatitis C is a viral disease triggered by an infection of the hepatitis C virus (HCV). If not recognized or treated, it can escalate into a chronic condition causing inflammation and possible liver damage. There are often no symptoms, so hepatitis C testing is critical. Screening will help determine HCV from other hepatitis causes, including [hepatitis A](#) and [hepatitis B](#).

An HCV antibody test can detect if you have been infected. It looks for antibodies to HCV in the blood.

If you are 18 or older, it is a good idea to get screened at least once for Hepatitis C. Testing is also recommended during pregnancy or if you have risk factors for HCV infection. Because acute hepatitis C often has no symptoms, screening is an important way to diagnose hepatitis C before it causes liver damage.

~~End of report~~

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REPORT

PT Name : MR. AAYUSH YADAV

Age : 23 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. DR. SANTOSH YADAV

Registered on : 02-09-2026 04:56 PM

MATOSHREE DIAGNOSTICS

Reg No : HL2501832638 / HYMH/KDV/MATOSHREE

Received on : 02-09-2026 06:37 PM

Barcode : HDM002404

Reported on : 02-09-2026 10:07 PM



INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

SAMPLE : Serum

HEPATITIS C VIRUS (HCV) ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS C ANTIBODY Method: ELISA	0.155	Index	0 - 1.00

NOTE-

- 1-False positive results are seen in Autoimmune diseases, Rheumatoid factor, Hypergammaglobulinemia, Paraproteinemia, passive antibody transfer, Anti- idiotypes & Anti superoxide dismutase
2. False negative results are seen in early Acute infection, Immunosuppression & Immuno-incompetence
3. HCV RNA PCR recommended in all Reactive results to differentiate between past and present infection

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : MR IMRAN PATEL

Ref By : DR. RAHUL T CHAKOR

Reg No : HL6001831923 / MH593

Barcode : HEN116971

INV : Hepatitis B Surface Antigen (HBs Ag) (Rapid)

Age : 46 Year | Sex : Male

Registered on : 31-08-2026 12:09 PM

Received on : 31-08-2026 01:28 PM

Reported on : 31-08-2026 02:39 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

HEPATITIS B SURFACE ANTIGEN (HBS AG) (RAPID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hepatitis B Surface Antigen (HBs Ag) ,Rapid Method: Immuno Chromatography	NEGATIVE	.	. - .

Interpretation :

HBsAg (Hepatitis B surface antigen) - A "positive" or "reactive" HBsAg test result means that the person is infected with hepatitis B. This test can detect the actual presence of the hepatitis B virus (called the "surface antigen") in your blood. If a person tests "positive," then further testing is needed to determine if this is a new "acute" infection or a "chronic" hepatitis B infection. A positive HBsAg test result means that you are infected and can spread the hepatitis B virus to others through your blood.

~~End of report~~

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REPORT

PT Name : MR. AAYUSH YADAV

Age : 23 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. DR. SANTOSH YADAV

Registered on : 02-09-2026 04:56 PM

MATOSHREE DIAGNOSTICS

Reg No : HL2501832638 / HYMH/KDV/MATOSHREE

Received on : 02-09-2026 06:37 PM

Barcode : HDM002404

Reported on : 03-09-2026 12:21 AM



INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

SAMPLE : Serum

HEPATITIS B SURFACE ANTIGEN (HBsAg)-ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hepatitis B Surface Antigen (HBsAg) Method: ELISA	0.16	Ratio	0 - 0.90
Reference Range Negative : < 0.90 Borderline : 0.9 - 1.1 Positive : > 1.1			

Clinical significance :

Classification of a hepatitis B infection requires the identification of several serological markers of the infection. The first marker to appear during the incubation phase is HBsAg, and indicates infection with HBV. Antibodies to HBsAg generally appears after HBsAg has been cleared from the blood stream, usually 6 months after infection, and its presence represents recovery and immunity. The presence of HBsAg antibodies should not be used as the sole marker in determining a prior hepatitis b infection. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, vaccination history, clinical examination and other findings.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : ANSH RAJPUT

Ref By : DR. OM HOSPITAL

Reg No : HL9201829645 / MH244

Barcode : HEI047425

INV : Hepatitis A Virus Total Antibodies (HAV-Total)

Age : 6 Year | Sex : Male

Registered on : 28-08-2026 08:47 PM

Received on : 29-08-2026 08:27 AM

Reported on : 29-08-2026 10:47 AM

SAMPLE COLLECTED AT :

HORMOCARE



SAMPLE : Serum

HEPATITIS A VIRUS TOTAL ANTIBODIES (HAV-TOTAL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HAV TOTAL Method: ELISA	0.45	Index	0 - 0.90
Reference Range			
Negative : < 0.90			
Borderline : 0.9 - 1.1			
Positive : >= 1.1			

~~End of report~~

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REPORT

PT Name : SHUBHAM DEVANE

Ref By : DR. SELF

Reg No : HL3901813667 / HYMH/BVI/H1944

Barcode : HEE125776

INV : Hepatitis A Virus IgM Antibodies

Age : 24 Year | Sex : Male

Registered on : 14-08-2026 01:53 PM

Received on : 14-08-2026 05:48 PM

Reported on : 14-08-2026 08:22 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

HEPATITIS A VIRUS IGM ANTIBODIES

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HAV -IgM Method: ELISA	0.16	Index	0 - 0.90
Reference Range			
Reference Range			
Negative : < 0.90			
Borderline : 0.9 - 1.1			
Positive : >= 1.1			

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : MRS.ARATI KANADE

Ref By : DR. VIJAY KANADE

Reg No : HL3601831721 / MH141

Barcode : HEN132319

INV : Hepatitis A Virus IgG Antibodies (HAV IgG)

Age : 45 Year | Sex : Female

Registered on : 31-08-2026 09:43 AM

Received on : 31-08-2026 10:31 PM

Reported on : 02-09-2026 09:09 AM

SAMPLE COLLECTED AT :

DEVANSH CLINICAL LABORATORY, WASHIM



SAMPLE : Serum

HEPATITIS A VIRUS IGG ANTIBODIES (HAV IGG)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS A VIRUS IGG Method: ELISA	0.32	OD Ratio	0 - 0.90
Reference Range			
Negative : < 0.90			
Borderline : 0.9 - 1.1			
Positive : >= 1.1			

~~End of report~~

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REPORT

PT Name : MR. ANIL CHAVAN

Ref By : DR. LIC

Reg No : HL4701830800 / MH005/MS103

Barcode : HEJ085144

INV : Hemoglobin

Age : 42 Year | Sex : Male

Registered on : 30-08-2026 08:13 AM

Received on : 30-08-2026 09:27 AM

Reported on : 30-08-2026 10:12 AM

SAMPLE COLLECTED AT :

SPECTRUM DIAG



SAMPLE : EDTA Blood

HEMOGLOBIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hemoglobin	13.9	g/dL	11 - 16

Interpreting test results

The numbers associated with your result are in grams per deciliter or g/dL. Hemoglobin reference ranges may vary depending on age and sex.

The high and low values of a normal test result, called the reference range, may differ depending on the laboratory and the methods used to conduct the test. For this reason, it is essential to look at the reference range listed on your test report.

An example of potential reference range, provided by the American Board of Internal Medicine, is listed in the following tables:

~~End of report~~

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REPORT

PT Name : HETVI VYAS
Ref By : DR. SELF
Reg No : HL5401832635 / HYMH/KDV/H6569
Barcode : HEE125358
INV : Glycated Haemoglobin (HbA1c)

Age : 25 Year | Sex : Female
Registered on : 02-09-2026 02:16 PM
Received on : 02-09-2026 05:20 PM
Reported on : 02-09-2026 07:06 PM

SAMPLE COLLECTED AT :
SHREE LAB



SAMPLE : EDTA Blood

GLYCATED HAEMOGLOBIN (HBA1C)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HBA1C Method: Fully Automated H.P.L.C.	5.4	%	0 - 6.5

BIOLOGICAL REFERENCE RANGES

Reference Range: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal 5.7% - 6.4% : Prediabetic ≥6.5% : Diabetic	Below 6.5% : Good Control 6.5% - 7% : Fair Control 7.0% - 8% : Unsatisfactory Control ≥8% : Poor Control

Estimated Average Glucose : 108.28 mg/dl . .

Method: CALCULATED

Reference Range

90 - 120 mg/dl : Good Control 121 - 150 mg/dl : Fair Control 151 - 180 mg/dl : Unsatisfactory Control > 180 mg/dl : Poor Control

Clinical significance :

Hemoglobin A1c (HbA1c) is a result of the nonenzymatic attachment of a hexose molecule to the N-terminal amino acid of the hemoglobin molecule. The attachment of the hexose molecule occurs continually over the entire life span of the erythrocyte and is dependent on blood glucose concentration and the duration of exposure of the erythrocyte to blood glucose. Therefore, the HbA1c level reflects the mean glucose concentration over the previous period (approximately 8-12 weeks, depending on the individual) and provides a much better indication of long-term glycemic control than blood and urinary glucose determinations. Diabetic patients with very high blood concentrations of glucose have from 2 to 3 times more HbA1c than normal individuals.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : SAURABH KOTHARI

Ref By : DR. SELF

Reg No : HL3301832634 / HYMH/KDV/H6569

Barcode : HEE125361

INV : HY FITNESS SMART 3

Age : 40 Year | Sex : Male

Registered on : 02-09-2026 02:12 PM

Received on : 02-09-2026 05:07 PM

Reported on : 02-09-2026 09:02 PM

SAMPLE COLLECTED AT :

SHREE LAB



SAMPLE : Fluoride Plasma-01

GLUCOSE F (BLOOD SUGAR FASTING)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GLUCOSE, FASTING (F) Method: GOD-POD	76.0	mg/dl	74.0 - 110.0

Interpretation

Depending on your treatment plan, you may check and record your blood sugar as many as four times a day or more often if you're taking insulin. Careful monitoring is the only way to make sure that your blood sugar level remains within your target range. People with type 2 diabetes who aren't taking insulin generally check their blood sugar much less frequently.

People who receive insulin therapy also may choose to monitor their blood sugar levels with a continuous glucose monitor. Although this technology hasn't yet completely replaced the glucose meter, it can significantly reduce the number of fingersticks necessary to check blood sugar and provide important information about trends in blood sugar levels.

Even with careful management, blood sugar levels can sometimes change unpredictably. With help from your diabetes treatment team, you'll learn how your blood sugar level changes in response to food, physical activity, medications, illness, alcohol, stress — and for women, fluctuations in hormone levels.

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~



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REPORT

PT Name : MR. RAM NAKSHATRA YADAV

Ref By : DR. SELF

Reg No : HL9201811060 / HYMH/KDV/H1886

Barcode : HEN173321

INV : Globulin

Age : 68 Year | Sex : Male

Registered on : 11-08-2026 06:02 PM

Received on : 11-08-2026 06:28 PM

Reported on : 11-08-2026 10:00 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

GLOBULIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GLOBULIN Method: CALCULATED	4.02	gm/dL	2.5 - 3.4

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : MRS.MOHANBAI JAIN

Age : 82 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. PRATHAMESH HOSPITAL

Registered on : 29-08-2026 03:28 PM

Reg No : HL2501830340 / MH372

Received on : 29-08-2026 06:24 PM

Barcode : HEN121162

Reported on : 29-08-2026 08:30 PM

INV : GGT (Gamma Glutamyl Transferase)

SAMPLE : Serum



GGT (GAMMA GLUTAMYL TRANSFERASE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Gamma GT Method: G glutamyl carboxy nitroanilide	95.2	U/L	5 - 55

Comments :

- The liver contains the highest levels of GGT, while the blood and some other organs contain minimal quantities.
- High levels of GGT in the blood could indicate that the enzyme is leaking out of the liver cells and into the blood, suggesting damage to the liver or bile ducts.
- A typical range for GGT levels in adults and children is between 0 and 30 international units per liter (IU/L). Newborn infants will have significantly higher levels right after birth.
- A person with a normal GGT test probably does not have liver disease. When someone who is in treatment for alcohol use disorder has a normal reading, this likely means that they have not had an alcoholic drink recently.
- GGT levels will rise according to the amount of liver damage a person has.

~~End of report~~



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REPORT

PT Name : MRINDI KAMBLE

Ref By : DR. H1944

Reg No : HL4401830996 / HYMH/BVI/H1944

Barcode : HEN173898

INV : FT4 (Thyroxine- Free)

Age : 20 Year | Sex : Female

Registered on : 30-08-2026 11:59 AM

Received on : 30-08-2026 06:02 PM

Reported on : 30-08-2026 08:45 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

FT4 (THYROXINE- FREE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Thyroxine - Free (FT4) Method: CLIA	1.15	ng/dL	0.89 - 1.76 Adults (21 - 87 Yrs) Pregnancy 0.7 - 2.0 : First Trimester 0.5 - 1.6 : 2nd and 3rd Tri

~~End of report~~



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REPORT

PT Name : MRINDI KAMBLE

Ref By : DR. H1944

Reg No : HL4401830996 / HYMH/BVI/H1944

Barcode : HEN173898

INV : FT3 (Trilodothyronine- Free)

Age : 20 Year | Sex : Female

Registered on : 30-08-2026 11:59 AM

Received on : 30-08-2026 06:02 PM

Reported on : 30-08-2026 08:45 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

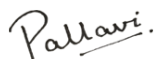
FT3 (TRILODOTHYRONINE- FREE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Trilodothyronine Free (FT3) Method: CLIA	3.25	pg/mL	2.3 - 4.2 Pregnancy 2.0 - 3.8

~~End of report~~



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Toll Free No. : 18001030287

REPORT

PT Name : MR. ANAND PRAKASH PANDEY

Ref By : DR. SELF

Reg No : HL9801832602 / HYMH/KDV/H1886

Barcode : HEN173358

INV : Erythrocyte Sedimentation Rate (ESR)

Age : 64 Year | Sex : Male

Registered on : 01-09-2026 04:01 PM

Received on : 01-09-2026 06:04 PM

Reported on : 01-09-2026 07:29 PM

SAMPLE COLLECTED AT :



SAMPLE : EDTA Blood

ERYTHROCYTE SEDIMENTATION RATE (ESR)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Erythrocyte sedimentation rate (ESR) Method: Wintrobe's	15	mm/Hour	0 - 20

NOTE

There are multiple causes of a high ESR test result. Some common conditions associated with a higher rate include:

- advanced age
- pregnancy
- anemia
- kidney disease
- obesity
- thyroid disease
- certain types of cancer, including some types of lymphoma and multiple myeloma

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : SANA

Ref By : DR. SELF

Reg No : HL9701832573 / RAJ41

Barcode : HEL002901

INV : Basic Fever Panel

Age : 30 Year | Sex : Female

Registered on : 31-08-2026 11:40 PM

Received on : 01-09-2026 04:10 AM

Reported on : 01-09-2026 07:17 AM

SAMPLE COLLECTED AT :



SAMPLE : EDTA Blood

COMPLETE BLOOD COUNT (CBC)+ESR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	11.0	g/dL	12.5 - 14.5	SLS-Hemoglobin
Total Red Blood Cell Count	4.18	10 ⁶ /uL	3.82 - 4.8	Electrical Impedance
Hematocrit (HCT)	31.9	%	36 - 46	Calculated
Total Leucocytes Count	5.62	10 ³ /uL	4 - 10	Electrical Impedance
Neutrophils Percentage	63.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	34.2	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	2.2	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	0.1	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.2	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	3.56	10 ³ /uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.92	10 ³ /uL	0.8 - 4.8	Calculated
Eosinophil-Absolute Count	0.12	10 ³ /uL	0.0 - 0.50	Calculated
Monocyte- Absolute Count	0.01	10 ³ /uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.01	10 ³ /uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV) Method: RBC HISTOGRAM	76.32	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	26.32	pg	27 - 32	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	34.48	g/dL	31.52 - 35.5	Calculated
Red Cell Distribution Width (RDW-CV)	15.0	%	12 - 15	Calculated
Platelet Count	232	10 ³ /uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	11.6	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	0.269	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	15.6	fL	15 - 18	Calculated
Erythrocyte Sedimentation Rate (ESR)	18	mm at end of 1st hr	00 - 20	Westergren

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



Corporate Office : HY PATHO LAB

📍 CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028

📞 7718962488

✉ info@hyatholab.in

🌐 www.hyatholab.in

☎ Toll Free No. : 18001030287

REPORT

PT Name : SAURABH KOTHARI
Ref By : DR. SELF
Reg No : HL3301832634 / HYMH/KDV/H6569
Barcode : HEE125360
INV : HY FITNESS SMART 3

Age : 40 Year | Sex : Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:31 PM
Reported on : 02-09-2026 06:10 PM

SAMPLE COLLECTED AT :
SHREE LAB



SAMPLE : EDTA Blood

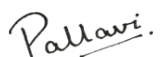
COMPLETE BLOOD COUNT (CBC)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	13.5	g/dL	13 - 18	SLS-Hemoglobin
Total Red Blood Cell Count	4.65	10 ⁶ /uL	4.0 - 5.5	Electrical Impedance
Hematocrit (HCT)	44.0	%	33 - 57	Calculated
Total Leucocytes Count	6.32	10 ³ /uL	4 - 10	Electrical Impedance
Neutrophils Percentage	68.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	22.3	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	3.9	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	5.2	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.3	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	4.31	10 ³ /uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.41	10 ³ /uL	0.8 - 4.8	Calculated
Eosinophils-Absolute Count	0.25	10 ³ /uL	0.0 - 0.50	Calculated
Monocyte-Absolute Count	0.33	10 ³ /uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.02	10 ³ /uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV)	94.62	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	29.03	pg	27.5 - 32.2	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	30.68	g/dL	29.4 - 34.5	Calculated
Red Cell Distribution Width (RDW-CV)	13.4	%	12 - 15	Calculated
Platelet Count	295	10 ³ /uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	9.7	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	0.286	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	11.2	fL	15 - 18	Calculated
Platelet larger cell Count (PLCC)	66	10 ³ /uL	13 - 126	Westergren
Platelet larger cell ratio (PLCR)	22.5	%	13 - 43	

~~End of report~~



Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist



Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



Corporate Office : HY PATHO LAB

CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028

7718962488

info@hyatholab.in

www.hyatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : SANA

Ref By : DR. SELF

Reg No : HL9701832573 / RAJ41

Barcode : HEL002901

INV : Basic Fever Panel

Age : 30 Year | Sex : Female

Registered on : 31-08-2026 11:40 PM

Received on : 01-09-2026 04:10 AM

Reported on : 01-09-2026 07:17 AM

SAMPLE COLLECTED AT :



SAMPLE : EDTA Blood

COMPLETE BLOOD COUNT (CBC)+ESR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	11.0	g/dL	12.5 - 14.5	SLS-Hemoglobin
Total Red Blood Cell Count	4.18	10 ⁶ /uL	3.82 - 4.8	Electrical Impedance
Hematocrit (HCT)	31.9	%	36 - 46	Calculated
Total Leucocytes Count	5.62	10 ³ /uL	4 - 10	Electrical Impedance
Neutrophils Percentage	63.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	34.2	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	2.2	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	0.1	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.2	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	3.56	10 ³ /uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.92	10 ³ /uL	0.8 - 4.8	Calculated
Eosinophil-Absolute Count	0.12	10 ³ /uL	0.0 - 0.50	Calculated
Monocyte- Absolute Count	0.01	10 ³ /uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.01	10 ³ /uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV) Method: RBC HISTOGRAM	76.32	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	26.32	pg	27 - 32	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	34.48	g/dL	31.52 - 35.5	Calculated
Red Cell Distribution Width (RDW-CV)	15.0	%	12 - 15	Calculated
Platelet Count	232	10 ³ /uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	11.6	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	0.269	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	15.6	fL	15 - 18	Calculated
Erythrocyte Sedimentation Rate (ESR)	18	mm at end of 1st hr	00 - 20	Westergren

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



Corporate Office : HY PATHO LAB

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7718962488

info@hypatholab.in

www.hypatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : MR.SHAMSH ALAM

Ref By : DR. SELF

Reg No : HL2001827149 / RAJ41/RAY10

Barcode : HEL095818

INV : Cholesterol LDL

Age : 45 Year | Sex : Male

Registered on : 26-08-2026 12:52 PM

Received on : 26-08-2026 05:05 PM

Reported on : 26-08-2026 07:03 PM

SAMPLE COLLECTED AT :

INDIAN CLINICAL LAB



SAMPLE : Serum

CHOLESTEROL LDL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LDL- Cholesterol Method: (PHOTOMETRY)	113.6	mg/dl	50 - 100 Desirable:100-129 Borderline High: 130-159 High: >160

~~End of report~~

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Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



Corporate Office : HY PATHO LAB

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7718962488

info@hypatholab.in

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Toll Free No. : 18001030287

REPORT

PT Name : TARUNNAM
Ref By : DR. SELF
Reg No : HL2201832577 / RAJ41
Barcode : HEL002911
INV : Anti Cyclic Citrullinated Peptide Antibody (Anti-CCP)

Age : 45 Year | Sex : Female
Registered on : 31-08-2026 11:47 PM
Received on : 01-09-2026 04:10 AM
Reported on : 01-09-2026 06:01 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

ANTI CYCLIC CITRULLINATED PEPTIDE ANTIBODY (ANTI-CCP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Anti CCP Method: CLIA	13.6	U/ml	0 - 20
Reference Range 20 Units or less: Negative 21-39 Units: Weak positive 40-59 Units: Moderate positive 60 Units or Greater: Strong positive			

Clinical significance :

Rheumatoid arthritis (RA) is a systemic autoimmune disease characterized by chronic joint inflammation that ultimately leads to joint destruction. RA affects approximately 1% of the world's population. The diagnosis of RA is established primarily on clinical criteria and serologic findings. Historically, rheumatoid factor (RF), which is an antibody specific for the Fc portion of human IgG, has been considered a marker for RA. RF is, in fact, one of the diagnostic criteria for RA that was established by the American College of Rheumatology.(1) Although 50% to 90% of patients with RA are RF-positive, the specificity of the RF test is known to be relatively poor. RF is found in many patients with other autoimmune diseases, infectious diseases and some healthy individuals. Consequently, a search for better diagnostic markers, with improved specificity for RA, ensued. Antiperinuclear factor (APF) and antikeratin antibodies (AKA), identified by immunofluorescence, were found to have a specificity of close to 90% for RA, but testing for these autoantibodies has never become popular. It was subsequently determined that APF and AKA react with the same antigen, specifically a citrullinated form of filaggrin (citrulline is an unusual amino acid formed by posttranslational modification of arginine residues by the enzyme peptidyl arginine deaminase).(2) Recombinant filaggrin fragments, after enzymatic deamination in vitro, react with autoantibodies in RA sera. Synthetic cyclic citrullinated peptide (CCP) variants also react with anti-filaggrin autoantibodies and serve as the substrate for detecting anti-CCP antibodies serologically. Most studies of anti-CCP antibodies demonstrated that these autoantibodies have much improved specificity for RA compared to RF.(3)

Please correlate with clinical conditions

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

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7718962488

info@hyatholab.in

www.hyatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : SAURABH KOTHARI

Age : 40 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. SELF

Registered on : 02-09-2026 02:12 PM

SHREE LAB

Reg No : HL3301832634 / HYMH/KDV/H6569

Received on : 02-09-2026 05:15 PM

Barcode : HEE125362

Reported on : 02-09-2026 10:48 PM



INV : HY FITNESS SMART 3

SAMPLE : Serum

C REACTIVE PROTEIN (CRP) QUANTITATIVE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
CRP	10.3	mg/dL	0 - 6.0

Method: (Immunoturbidimetry)

Comments

CRP elevations are nonspecific and may be useful for the detection of systemic inflammatory processes; to assess treatment of bacterial infections with antibiotics; to detect intrauterine infections with concomitant premature amniorrhexis; to differentiate between active and inactive forms of disease with concurrent infection, eg, in patients suffering from systemic lupus erythematosus or colitis ulcerosa; to therapeutically monitor rheumatic disease and assess anti-inflammatory therapy; to determine the presence of postoperative complications at an early stage, such as infected wounds, thrombosis, and pneumonia; and to distinguish between infection and bone marrow rejection. Postoperative monitoring of CRP levels of patients can aid in the recognition of unexpected complications (persisting high or increasing levels).

Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. It also allows judgments about the disease genesis. Persistence of a high serum CRP concentration is usually a grave prognostic sign that generally indicates the presence of an uncontrolled infection.

Please correlate with clinical conditions.

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



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7718962488

info@hypatholab.in

www.hypatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : MRS.PRIYA MALI8

Age : 42 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. NEETI UPASANI

Registered on : 31-08-2026 05:02 PM

Reg No : HL4501832446 / MH466

Received on : 31-08-2026 05:11 PM

Barcode : HEL067129

Reported on : 31-08-2026 07:48 PM



INV : BUN (Blood Urea Nitrogen)

SAMPLE : Serum

BUN (BLOOD UREA NITROGEN)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
BUN (Blood Urea Nitrogen) Method: CALCULATED	8.71	mg/dl	7 - 25

~~End of report~~

NRahate

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Pallavi.

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Consultant Pathologist (Reg. 0375)



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7718962488

info@hypatholab.in

www.hypatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : MRS MADHAVI

Age : 49 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. SELF

Registered on : 31-08-2026 04:56 PM

Reg No : HL3101832432 / RAJ53

Received on : 31-08-2026 05:13 PM

Barcode : HEN121673

Reported on : 31-08-2026 07:29 PM

INV : Bilirubin (Total + Direct + Indirect)



SAMPLE : Serum

BILIRUBIN (TOTAL + DIRECT + INDIRECT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
BILIRUBIN - TOTAL Method: DIAZO	0.37	mg/dl	0 - 1.2
BILIRUBIN -DIRECT Method: DIAZO	0.15	mg/dl	0 - 0.4
BILIRUBIN (INDIRECT) Method: CALCULATED	0.22	mg/dl	0 - 0.9

Clinical significance :

Bilirubin is one of the most commonly used tests to assess liver function. Approximately 85% of the total bilirubin produced is derived from the heme moiety of hemoglobin, while the remaining 15% is produced from the red blood cell precursors destroyed in the bone marrow and from the catabolism of other heme-containing proteins. After production in peripheral tissues, bilirubin is rapidly taken up by hepatocytes where it is conjugated with glucuronic acid to produce mono- and diglucuronide, which are excreted in the bile.

A number of inherited and acquired diseases affect 1 or more of the steps involved in the production, uptake, storage, metabolism, and excretion of bilirubin. Bilirubinemia is a frequent and direct result of these disturbances.

Jaundice can occur as a result of problems at each step in the metabolic pathway. Disorders may be classified as those due to: increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice).

The most commonly occurring form of unconjugated hyperbilirubinemia is that seen in newborns and referred to as physiological jaundice. Elevated unconjugated bilirubin in the neonatal period may result in brain damage (kernicterus). Treatment options are phototherapy and, if severe, exchange transfusion.

The rare genetic disorders, Crigler-Najjar syndromes type I and type II, are caused by a low or absent activity of bilirubin UDP-glucuronyl-transferase. In type I, the enzyme activity is totally absent, the excretion rate of bilirubin is greatly reduced and the serum concentration of unconjugated bilirubin is greatly increased. Patients with this disease may die in infancy owing to the development of kernicterus.

The increased production of bilirubin, that accompanies the premature breakdown of erythrocytes and ineffective erythropoiesis, results in hyperbilirubinemia in the absence of any liver abnormality.

Please correlate with clinical conditions.

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)